

BEACOPP

BEACOPP is a combination of drugs used to treat Hodgkin lymphoma.

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What is BEACOPP?

BEACOPP is a combination of chemotherapy drugs. It is used to treat Hodgkin lymphoma.

It is best to read this information with our general information about [chemotherapy](#) and [Hodgkin lymphoma](#)

BEACOPP is named after the initials of the drugs used:

- B – [bleomycin](#)
- E – [etoposide](#)
- A – [doxorubicin](#) (also called Adriamycin[®])
- C – [cyclophosphamide](#)
- O – [vincristine](#) (also called Oncovin[®])
- P – [procarbazine](#)
- P – prednisolone, a [steroid](#).

BEACOPP is also known as BEACOPP-14.

Escalated BEACOPP uses the same drugs, but in different doses.

BEACOPP is very similar to a combination of chemotherapy called [BEACOPDac](#) and escalated BEACOPDac. These use most of the same drugs in different doses, as well as a drug called [dacarbazine](#).

High doses of cyclophosphamide can sometimes irritate the lining of your bladder. A drug called [mesna](#) helps to protect your bladder from the irritation. It is usually given with escalated BEACOPP, and sometimes with BEACOPP.

Your cancer team will talk to you about this treatment and its possible side effects before you agree ([consent](#)) to have treatment.

More information about this treatment

This information is correct at time of publishing. But sometimes the types of cancer this treatment is used for, or treatment side effects, may change between revision dates.

You can talk to your cancer team if you want more detailed information about this treatment. Or visit the [electronic Medicines Compendium \(eMC\)](#) website, which has patient information leaflets (PIL) for individual drugs.

How BEACOPP is given

You usually have BEACOPP in a chemotherapy day unit or during a stay in hospital.

During your course of treatment, you will meet someone from your cancer team, such as a:

- cancer doctor
- chemotherapy nurse or specialist nurse
- specialist pharmacist.

This is who we mean when we mention doctor, nurse or pharmacist in this information.

Before, or on day 1 and day 8 of treatment, you will have a blood test. This is to check that it is safe for you to have chemotherapy.

You will meet with a doctor, nurse or pharmacist before you have treatment. They will talk to you about your blood results and ask how you have been feeling. If your blood results are okay, the pharmacy team will prepare your chemotherapy.

Your nurse will usually give you anti-sickness drugs before chemotherapy. You will have some of your chemotherapy drugs through 1 of the following:

- a [cannula](#) – a short, thin tube the nurse puts into a vein in your arm or hand
- a [central line](#) – a fine tube that goes under the skin of your chest and into a vein close by
- a [PICC line](#) – a fine tube that is put into a vein in your arm and goes up into a vein in your chest
- an [implantable port](#) (portacath) – a disc that is put under the skin on your chest or arm and goes into a vein in your chest.

Procarbazine and prednisolone are given as capsules and tablets that you swallow. Doxorubicin is given as a slow injection into your line or cannula. It may be given along with a drip (infusion) of fluids to flush it through. Cyclophosphamide is given as a slow injection or as a drip. Etoposide, bleomycin and vincristine are all given as drips.

Your cancer team may also give you anti-sickness drugs and other medicines to take home. Take all your capsules or tablets exactly as they tell you to.

Your course of chemotherapy

Each cycle of BEACOPP usually takes 14 days (2 weeks). Each cycle of escalated BEACOPP takes 21 days (3 weeks). A cycle includes the days you have the BEACOPP drugs followed by a rest period.

You may have between 2 and 8 cycles. Your nurse or doctor will discuss your treatment plan with you. They may give you a copy of your treatment plan to take home.

- On day 1 the nurse will give you doxorubicin, cyclophosphamide and etoposide. If you are having escalated BEACOPP you will have [mesna](#). You will also start taking procarbazine and prednisolone.
- On days 2 and 3 the nurse will give you etoposide.
- On day 8 the nurse will give you bleomycin and vincristine.

You take procarbazine on days 1-7. If you are having BEACOPP you take prednisolone on days 1-7. If you are having escalated BEACOPP you take prednisolone on days 1-14. Your nurse or pharmacist will explain when to take the tablets.

After you finish all your prednisolone tablets, you have a rest period. This means you do not take any tablets for the next 7 days. After the rest period has finished, you start the next cycle. This is the same as the first cycle.

Taking your capsules and tablets

Procarbazine

Your doctor, nurse or pharmacist may advise you to avoid some types of foods and alcohol while taking procarbazine. These include:

- mature cheese
- salami
- Oxo[®], Bovril[®], Marmite[®] and other beef or yeast extracts
- alcohol
- non-alcoholic beers and wines.

This is because they can react with procarbazine and cause side effects including:

- feeling sick
- headaches
- sweating or flushing
- drowsiness
- breathing problems.

This type of reaction is not common. But if you notice any of these symptoms, contact the hospital for advice.

Procarbazine capsules must be swallowed whole with a glass of water. Do not chew, open or crush them. Take them at the same time every day on the days you have been told to.

If you forget to take the capsules, contact your doctor or nurse for advice. Do not take a double dose.

Other things to remember about your capsules:

- Wash your hands after taking your capsules.
- Other people should avoid direct contact with the chemotherapy drugs.
- Keep them in the original package and at room temperature, away from heat and direct sunlight.
- Keep them safe and out of sight and reach of children.
- If you are sick just after taking the capsules, contact the hospital. Do not take another dose.
- If your treatment is stopped, return any unused capsules to the pharmacist.

Prednisolone

The prednisolone tablets should be taken after breakfast. This helps to reduce any risk of indigestion or stomach problems. It is best to take prednisolone in the morning rather than as separate doses through the day. If you take it later in the day, it may keep you awake at night.

About side effects

We explain the most common side effects of this treatment here. We also include some that are less common.

You may get some of the side effects we mention, but you are unlikely to get all of them. And you may have some side effects, including rarer ones, that we have not listed here.

Always tell your doctor, nurse or pharmacist about any side effects you have. They can give you:

- drugs to help control some side effects
- advice about managing side effects.

It is important to take any drugs exactly as explained. This means they will be more likely to work for you.

Serious and life-threatening side effects

Some cancer treatments can cause serious side effects. Sometimes, these may be life-threatening. Your doctor, nurse or pharmacist can explain the risk of these side effects to you.

Contact the hospital

Your doctor, nurse or pharmacist will give you 24-hour contact numbers for the hospital. If you feel unwell or need advice, you can call at any time of the day or night. Save these numbers in your phone or keep them somewhere safe.

Side effects while treatment is being given

Some people may have side effects while they are being given the chemotherapy into a vein or shortly after they have it.

Allergic reaction

Some people have an allergic reaction while having this treatment. Signs of a reaction can include:

- feeling hot or flushed
- shivering
- itching
- a skin rash
- feeling dizzy or sick
- a headache
- feeling breathless or wheezy
- swelling of your face or mouth
- pain in your back, tummy or chest.

Your nurse will check you for signs of a reaction during your treatment. If you feel unwell or have any of these signs, tell them straight away. If you do have a reaction, they can treat it quickly.

Sometimes a reaction happens a few hours after treatment. If you develop any of these signs or feel unwell after you get home, contact the hospital straight away on the 24-hour number.

Always call 999 if swelling happens suddenly or you are struggling to breathe.

The drug leaks outside the vein

Sometimes cancer drugs that are given into a vein may leak outside the vein. If this happens, some drugs can damage the tissue near the vein. This is called extravasation. Extravasation is not common, but it is important that it is dealt with quickly. If you have any of the following symptoms around the vein during or after your treatment, tell your nurse straight away:

- stinging
- pain
- swelling
- if you have white skin, the area may become red
- if you have black or brown skin, the area may become darker.

Very common side effects

These side effects happen to 10 or more people in every 100 people (10% or more) who have this treatment.

Risk of infection

This treatment can reduce the number of white blood cells in your blood. These cells fight [infection](#). If the number of white blood cells is low, you are more likely to get an infection. A low white blood cell count is called neutropenia.

Your doctor may give you antibiotics and other drugs to try to stop you getting an infection. These are called prophylactic medicines.

You will have a drug called [G-CSF](#) on days 9 to 13 of each cycle of your treatment. This encourages the body to make more white blood cells. You have it as a small injection under the skin.

An infection can be very serious when the number of white blood cells is low. It is important to get any infection treated as soon as possible. Contact the hospital straight away on the 24-hour contact number you have been given if:

- your temperature goes over 37.5°C (99.5°F)
- you suddenly feel unwell, even with a normal temperature
- you have symptoms of an infection
- your temperature goes below 36°C (96.8°F).

Symptoms of an infection include:

- feeling shivery and shaking
- a sore throat
- a cough
- breathlessness
- diarrhoea
- needing to pass urine (pee) a lot, or discomfort when you pass urine.

It is important to follow any specific advice your cancer treatment team gives you.

The number of white blood cells will usually return to normal before your next treatment. You will have a blood test before having more treatment. If your white blood cell count is low, your doctor may delay your treatment for a short time, until your cell count increases.

Bruising and bleeding

This treatment can reduce the number of platelets in your blood. Platelets are cells that help the blood to clot.

If the number of platelets is low, you may bruise or bleed easily. You may have:

- nosebleeds

- bleeding gums
- heavy periods
- blood in your urine (pee) or stools (poo)
- tiny red, brown or purple spots that may look like a rash – these spots can be harder to see if you have black or brown skin.

If you have any unexplained bruising or bleeding, contact the hospital straight away on the 24-hour number. You may need a drip to give you extra platelets. This is called a platelet transfusion.

Anaemia (low number of red blood cells)

This treatment can reduce the number of red blood cells in your blood. Red blood cells carry oxygen around the body. If the number of red blood cells is low, this is called anaemia. You may feel:

- very low in energy
- breathless
- dizzy and light-headed.

If you have these symptoms, contact the hospital straight away on the 24-hour number. You may need treatment for anaemia. If you are very anaemic, you may need a drip to give you extra red blood cells. This is called a blood transfusion.

Feeling sick

Your doctor, nurse or pharmacist will prescribe anti-sickness drugs to help prevent or control sickness. Take the drugs exactly as they tell you to, even if you do not feel sick. It is easier to prevent sickness than to treat it after it has started.

If you feel sick, take small sips of fluid often and eat small amounts regularly. It is important to drink enough fluids. If you continue to feel sick, or if you are sick (vomit) 1 to 2 times in 24 hours, contact the hospital on the 24-hour number as soon as possible. They will give you advice. They may change your anti-sickness treatment. Let them know if you still feel sick.

Changes in appetite

This treatment can affect your appetite. You may feel like you don't want to eat much. Or, while you are taking prednisolone, you may feel hungry and eat more than usual.

Do not worry if your appetite changes for a few days. But tell your doctor or nurse if:

- you have indigestion – you may need medicines to help control this while you are taking prednisolone
- your appetite does not come back after a few days – they may give you food or drink supplements or suggest changes to your diet or eating habits to help.

Tummy pain

You may get pain in your tummy (abdomen), or have indigestion. Your doctor, nurse or pharmacist can give you advice or treatment to help. Contact the hospital straight away on the 24-hour number if your symptoms do not go away, or get worse.

Sore mouth and throat

This treatment may cause a [sore mouth and throat](#). You may also get mouth ulcers. This can make you more likely to get a mouth or throat infection. Use a soft toothbrush to clean your teeth or dentures in the morning, at night and after meals.

Contact the hospital straight away on the 24-hour number, if:

- a sore mouth or throat affects how much you can drink or eat
- your mouth, tongue, throat or lips have any blisters, ulcers or white patches.

They can give you advice, and mouthwash or medicines to help with the pain or to treat any infection. Follow their advice and make sure you:

- drink plenty of fluids
- avoid alcohol and tobacco
- avoid food or drinks that irritate your mouth and throat.

Constipation

This treatment can cause [constipation](#). Constipation means that you are not able to pass stools (poo) as often as you normally do. It can become difficult or painful. Here are some tips that may help:

- Drink at least 2 litres (3½ pints) of fluids each day.
- [Eat high-fibre foods](#), such as fruit, vegetables and wholemeal bread.
- Do regular gentle exercise, like going for short walks.

If you have constipation, contact the hospital on the 24-hour number for advice. They can give you drugs called laxatives to help.

If you have not been able to pass stools for over 2 days and are being sick, contact the 24-hour number straight away.

Diarrhoea

This treatment may cause [diarrhoea](#). Diarrhoea means passing more stools (poo) than is normal for you, or having watery or loose stools. You may also have stomach cramps. If you have a stoma, it may be more active than usual.

If you are passing loose stools 3 or more times a day and this is not normal for you, contact the hospital as soon as possible on the 24-hour number. Follow the advice they give you about:

- taking anti-diarrhoea medicines
- drinking enough fluids to keep you hydrated and to replace lost salts and minerals
- any changes to your diet that might help.

They might also ask you for a specimen of your stool to check for infection.

Red or pink urine (pee)

After doxorubicin, your urine may be pink or red for up to 48 hours. This is because of the red colour of the drug. It is not harmful. But always check with your doctor, nurse or pharmacist if you are worried.

Hair loss

Your hair will get thinner. Or you may lose all the hair from your head. You may also lose your eyelashes and eyebrows, as well as other body hair. Hair loss usually starts after your first or second treatment.

If you want to cover up hair loss, there are different ways you can do this. Your nurse will give you information about [coping with hair loss](#).

Remember to [protect your skin from the sun](#). Use suncream with a sun protection factor (SPF) of at least 30 on your scalp. Or cover up with a hat or scarf.

Hair loss is almost always temporary. Your hair will usually grow back after treatment ends.

Muscle or joint pain

You may get pain in your muscles or joints for a few days after treatment. If this happens, tell your doctor, nurse or pharmacist. They can give you painkillers and advice. They can also tell you if any of the painkillers you usually take are suitable.

If you have muscle or joint pain, try:

- placing a heat pad or covered hot water bottle against the painful area
- taking warm baths
- planning your activities to include regular rests.

Jaw pain

Vincristine may cause pain in your jaw. Tell your doctor, nurse or pharmacist if you notice this.

Numb or tingling hands or feet (peripheral neuropathy)

This treatment may affect the nerves in your fingers and toes. This can cause numbness, tingling or pain in your hands or feet. This is called [peripheral neuropathy](#). You might find it hard to do fiddly tasks such as fastening buttons or tying shoelaces.

If you have these symptoms, always tell your doctor, nurse or pharmacist. They sometimes need to change the drug or the dose of the drug. The symptoms usually improve slowly after treatment ends. But for some people they continue and are a long-term side effect of treatment.

Skin changes

Chemotherapy can affect your skin. If your skin feels dry, try using soap-free cleansers and unperfumed moisturising cream every day.

This treatment can also:

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- cause a rash, which may be itchy
- make your skin darker in some areas
- make any area treated with radiotherapy become red or sore. If you have white skin the area will become red and if you have black or brown skin the area might become darker.
- make you more sensitive to the sun.

Your [skin may burn more easily](#) during treatment and for several months after. Use a sun cream of at least SPF 30. SPF stands for sun protection factor. Cover up with clothing and wear a hat.

Always tell your doctor, nurse or pharmacist about any [skin changes](#). They can give you advice or prescribe creams or medicines to help. Changes to your skin are usually temporary and improve when treatment ends.

Serious skin changes

Rarely, this treatment can cause a serious skin reaction that needs to be treated immediately in hospital. Contact the hospital straight away on the 24-hour number if you have any of these symptoms:

- a skin rash that is spreading
- blistering or peeling skin
- flu-like symptoms, such as a high temperature and joint pain
- sores on your lips or in your mouth.

Nail changes

This treatment can affect [your nails](#). They may grow more slowly or break more easily. You might notice ridges or white or dark lines across your nails. These changes usually disappear as the nails grow out after treatment. Sometimes nails can become loose or fall off. Tell your doctor, nurse or pharmacist about any changes to your nails.

If the skin around your nails becomes sore and swollen, contact the hospital straight away on the 24-hour number. These might be signs of an infection.

Sore eyes

Your eyes may become watery and feel sore. Your doctor can prescribe eye drops to help with this. If your eyes become red and inflamed, tell your doctor. This is called conjunctivitis. You may need eye drops to treat it.

Tumour lysis syndrome (TLS)

Some people are at risk of developing a condition called tumour lysis syndrome (TLS). The risk is highest at the beginning of treatment. TLS happens when treatment makes large numbers of cancer cells die and break down quickly. This releases lots of waste products into the blood and can affect the kidneys and heart.

TLS can be prevented or treated. You will have regular blood tests to check for TLS. You may have:

- extra fluids through a drip

- medicines such as allopurinol tablets or [rasburicase](#) through a drip.

Drinking at least 2 litres (3½ pints) of fluid a day will also help.

Effects on the heart

This treatment can affect how the heart works. You may have [tests](#) to check how well your heart is working. These may be done before, during and after treatment.

If the treatment is causing [heart problems](#), your doctor may change the type of treatment you are having.

Contact the hospital straight away on the 24-hour number if you have any of these symptoms during or after treatment:

- breathlessness
- dizziness
- changes to your heartbeat (palpitations)
- swollen feet and ankles.

Other conditions can cause these symptoms, but it is important to get them checked by a doctor.

Always call **999** if you have:

- chest pain, pressure, heaviness, tightness or squeezing across the chest
- difficulty breathing.

Effects on the liver and kidneys

This treatment can affect how your kidneys or liver work. This is usually mild and goes back to normal after treatment ends. You will have blood tests to check how well your kidneys and liver are working.

Contact the hospital on the 24-hour number if you:

- have blood in your urine (pee)
- are passing less urine or peeing less often than usual.

Drinking fluids also helps protect your kidneys. The advice is usually to try to drink at least 2 litres (3½ pints) of fluid each day. But follow any advice from your doctor, nurse or pharmacist about how much is right for you.

Effects on the kidneys and liver

This treatment may affect how your kidneys or liver work. This is usually mild. You will have blood tests to check how well your kidneys and liver are working.

High blood pressure

This treatment can cause high blood pressure. Tell your doctor or nurse if you have ever had any problems with your blood pressure. Your nurse will check it regularly during your treatment.

Raised blood sugar levels

This treatment may raise your blood sugar levels and increase the risk of diabetes.

You will have regular blood tests to check this.

Contact the hospital on the 24-hour number if you have these symptoms during treatment or after it ends:

- feeling thirsty all the time
- needing to pass urine (pee) more often than usual
- feeling more tired than usual.
- losing weight without trying.

If you have diabetes, your blood sugar levels may be higher than usual. You may need advice and support from your GP or diabetes team about managing this.

Less common side effects

These side effects happen to less than 10 in 100 people (less than 10%) who have this treatment. Some of them are much rarer than this but they are still important to know about. Rare means a side effect that happens to less than 1 in 1,000 people (less than 0.1%).

Effects on the lungs

This treatment can cause changes to the lungs. You may have tests to check your lungs before and during treatment. Contact the hospital straight away on the 24-hour number if you develop:

- a cough that does not go away
- wheezing
- breathlessness.

You should also tell them if any existing breathing problems get worse.

Smoking increases your risk of lung problems. If you smoke, ask your doctor, nurse or pharmacist for advice about stopping.

After treatment with bleomycin, breathing in high doses of oxygen can cause lung problems. If you need to have a general anaesthetic or oxygen therapy for any reason, always tell the doctor that you have had bleomycin. Some people choose to wear a medical alert identifier.

You should not scuba dive for 1 year after treatment with bleomycin. After this, you should have tests to check whether scuba diving is safe for you. Your cancer doctor can give you more information about this.

Effects on the brain

Rarely, this treatment causes a brain condition that can be serious. You can make a full recovery from this. But it must be diagnosed and treated quickly.

This condition can cause:

- a headache that does not get better
- drowsiness or confusion
- changes in eyesight
- fits (seizures).

If you have any of these symptoms, it is important to either:

- contact the hospital straight away on the 24-hour number
- go to the hospital straight away.

You should not drive yourself to hospital.

Second cancer

BEACOPP can increase the risk of developing a second cancer or a blood disorder called myelodysplastic syndrome years later. This is rare. The benefits of treatment usually far outweigh this risk. Your doctor can talk to you about this.

Food and alcohol interactions with procarbazine

When taking procarbazine, you should avoid:

- mature cheese
- salami
- Oxo[®], Bovril[®], Marmite[®] and other beef or yeast extracts
- alcohol
- non-alcoholic beers and wines.

This is because they can cause a reaction. Signs of a reaction include:

- feeling sick
- headaches
- sweating or flushing
- drowsiness
- breathing problems.

This type of reaction is not common. But if you notice any of these symptoms, contact the hospital for advice. Your nurse, doctor or pharmacist can give you more advice on types of food and drink to avoid.

Other information

Blood clot risk

Cancer and some cancer treatments can increase the risk of a [blood clot](#). Contact the hospital straight away on the 24-hour number if you have any of these symptoms during or after treatment:

- throbbing pain or swelling in a leg or arm
- reddening of the skin in the area – if you have black or brown skin, this can be harder to notice, but the skin might become darker
- suddenly feeling breathless or coughing.

Always call **999** if you have:

- chest pain
- difficulty breathing.

A blood clot is serious, but it can be treated with drugs called anticoagulants. These thin the blood. Your doctor, nurse or pharmacist can give you more information about preventing and treating blood clots.

Blood irradiation

If you have or have had Hodgkin lymphoma, any blood and platelets you are given should first be treated with radiation (irradiation). This lowers the risk of the donated blood cells reacting against your own. It will not damage the blood or make you radioactive.

Your doctor will record in your medical notes that you should only be given irradiated blood products. They will also give you a card to carry in case you are treated at another hospital. Keep this card with you at all times and remind your hospital team that you need irradiated blood or platelets.

Other medicines

Some medicines can affect how this treatment works or be harmful while you are having it. Always tell your cancer doctor, nurse or pharmacist about any drugs you are taking or planning to take, such as:

- medicines you have been prescribed
- medicines you buy in a shop, pharmacy or online
- vitamins or supplements
- [herbal drugs and complementary or homeopathic therapies](#)
- recreational drugs – for example, cannabis.

Vaccinations

Cancer doctors usually recommend that people with cancer have vaccinations for flu and [coronavirus \(covid\)](#). They may also recommend other vaccines, such as Shingrix[®] for shingles. These all help reduce your risk of serious illness from these infections. Most people can have these vaccines, including people with weak immune systems.

You should not have live vaccines if your immune system is weak. This includes if you are having or recently had chemotherapy, radiotherapy or other cancer treatments that affect your immune system. Live vaccines can make you unwell because they contain a very weak version of the illness they protect you against. There are several live vaccines, including the yellow fever vaccine.

It is important to ask your doctor, nurse or pharmacist for advice about having vaccinations. They can explain what vaccines are right for you and when it is best to have them.

Breastfeeding

You are advised not to [breastfeed](#) while having this treatment, or for some time after treatment ends. This is because the drugs could be passed to the baby through breast milk.

Your doctor, nurse or pharmacist can give you more information.

Fertility

Some cancer drugs can affect whether you can [get pregnant](#) or [make someone pregnant](#). If you are worried about this, it is important to talk with your doctor before you start treatment.

Changes to periods

If you have periods, these may become irregular or stop while you are having this treatment. They might return after treatment, but this does not always happen. Your [menopause](#) may start sooner than it would have done. Your doctor, nurse or pharmacist can give you more information.

Sex

It is possible that small amounts of chemotherapy may be passed on through vaginal fluids or semen. If you have sex in the first few days after treatment, your cancer team will usually advise using condoms or a dental dam to protect your partner.

Medical and dental treatment

If you need medical treatment for any reason other than cancer, always tell the healthcare professional that you are having cancer treatment. Give them the contact details for your cancer doctor or cancer team so they can ask for advice.

If you have appointments with a dentist, always tell them you are having cancer treatment. Talk to your cancer team before you have any dental treatment.

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Visit the [electronic Medicines Compendium \(eMC\)](#) to download a Patient Information Leaflet (PIL) for more detailed information. The leaflet lists all known side effects.

[Cancer Information Development team](#)

Author

Date reviewed

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Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

How to print a PDF from this website

Instructions on how you can make a PDF version of this page using the print function on your browser.

Clinical Information Nurse Specialists

Our Cancer Information Nurse Specialists are dedicated cancer nurses available to talk to on our Macmillan Cancer Support Line.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help